

Having an operation for breast cancer

Your preparation for surgery and recovery afterwards will depend on the type of operation you have.

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Before your operation

Before your operation for [breast cancer](#), you may go to a pre-operative assessment clinic. At the clinic, you may have tests to check your general health. These can include:

- blood tests
- a chest x-ray
- a recording of your heart (ECG).

Your surgeon or breast care nurse will talk to you about how your [breast or chest will look after your surgery](#). They may show you photographs of other people who have had surgery for breast cancer. They may also put you in contact with someone who has had the same operation. You could also contact a local support group or [Breast Cancer Now](#), or visit our [breast cancer forum](#) to talk with people who have been affected by breast cancer, share your experience, and ask an expert your questions.

You will usually go into hospital on the day of your operation. You will meet the doctor who gives you the anaesthetic. They are called an anaesthetist. The nurses may give you elastic stockings to wear during and after the operation. These are called TED stockings. They help prevent blood clots.

We have more information about [preparing for surgery](#).

After your operation

Your recovery after surgery will depend on the type of operation you have.

You can usually go home the same day as breast cancer surgery or the following day.

If you have breast reconstruction at the same time as a mastectomy, you will stay in hospital for 1 to 5 days. This will depend on the type of reconstruction you have.

Your healthcare team will encourage you to start moving around as soon as possible after your operation. This can help reduce the risk of problems that can happen after surgery.

Your wound

You will usually have a dressing covering your wound. This may not be removed for the first few days after your operation. Before you go home, the nurses will tell you how to look after it.

After the operation, it is common to have some swelling and bruising around the wound. This should improve after a few weeks. If it does not, tell your breast care nurse. If you had an SLNB, you may see the blue dye in your skin for a few weeks or months. This is normal.

How long it takes for the wound to heal depends on the operation you had. Your wound may be closed with glue or stitches that dissolve. These do not need to be removed. If you do not have stitches that dissolve, they are usually removed 7 to 10 days after your operation. You can arrange this with your practice or district nurses at your GP surgery . Or it may happen at your outpatient appointment.

Drains

You may have a long, thin plastic drainage tube coming from your wound. This is attached to a drainage bag or bottle. Fluid from the wound drains into the bag or bottle.

The tube is usually left in for a few days. You can go home with it still in place. A practice nurse or a district nurse may check and remove it when you are at home. Or you might have it checked and removed at the hospital.

Pain

You will probably have some pain or discomfort around the wound. If you had lymph nodes removed, you may also have some pain or discomfort in your armpit.

It can help to take painkillers regularly until the pain starts to improve. This usually takes a few days. The nurses on the hospital ward will usually give you the painkiller to take. If you had a mastectomy, you may need painkillers for 1 or 2 weeks. Tell your cancer doctor or breast care nurse if the painkillers are not helping. They may be able to prescribe different ones for you to try.

Possible problems after surgery

Wound infection

Signs of infection can include:

- warmth around the wound
- redness around the wound
- swelling around the wound or discharge coming from it
- feeling unwell with a fever

Tell your breast care nurse, cancer doctor or GP if you get any of these symptoms, even after you go home.

Fluid collecting around the wound (seroma)

A seroma is a soft bulge or swelling around the wound or very close to it. It is caused by a build-up of fluid. It usually goes away within a few weeks. Talk to your surgeon or breast care nurse if the swelling does not go away. They may drain the fluid with a needle and syringe. This may cause a little discomfort when it is being done. And it may need to be repeated if the fluid builds up again.

Stiff shoulder or arm

If you had a mastectomy or lymph nodes removed, your shoulder or arm may feel sore or stiff.

A physiotherapist or nurse will show you some arm exercises to do. These will help improve the movement in your shoulder and arm. They will also reduce the risk of long-term problems. You should start the exercises the day after your operation and slowly build up what you do. It is important to keep doing them until you can move your arm as well as you could before your operation.

Some people find it helps to take painkillers 1 hour before doing the exercises.

Breast Cancer Now has a [leaflet about these exercises](#).

Numbness and tingling in the upper arm

You may have numbness or a tingling feeling in your upper arm. This is more likely if you had all the lymph nodes in your armpit removed.

Numbness and tingling in the upper arm is caused by swelling and damage to the nerves in your breast and armpit. This can happen during or after the operation. It may slowly improve over a few months, but numbness can sometimes be permanent. Talk to your surgeon or nurse if you are worried.

Cording

If you had [surgery to remove lymph nodes](#) in the armpit, you may develop cording. This is also called axillary web syndrome. It feels like a tight cord going from your armpit down the inside of your arm. You may be able to see the cord as well as feel it. Sometimes there is more than 1 cord. It can feel tight and painful and can affect the movement in your arm and shoulder.

Cording may happen days or weeks after surgery, or sometimes months later. It is less likely to happen if you only had 1 or 2 lymph nodes removed. It is not clear exactly what causes cording. It may be caused by changes in the lymphatic vessels after surgery.

If you are worried, ask your breast care nurse for advice. Cording usually gets better on its own. But you may need to visit a physiotherapist. They can help you with exercises to stretch the cord and improve your movement.

We have more information about cording in our information on the [late effects of breast cancer treatment](#).

When you get home

Your recovery will depend on the type of operation you have. You may need to avoid lifting or carrying anything heavy for a few weeks.

If you drive, contact your car insurance company to let them know you have had an operation. Most people are ready to drive about 4 weeks after their operation. But some insurance policies give specific time limits for not driving after surgery. Do not drive unless you feel in full control of the car.

When you are home, it is important to follow the advice you were given by your breast care nurse. You should keep doing the exercises you were given in hospital and try do some [light exercise](#), such as walking. This can help to build up your energy so you can gradually get back to your normal activities.

Outpatient appointment and results

You will have a clinic appointment to see your surgeon and breast care nurse. They will check that the wound is healing properly. They will also tell you about the tissue that was removed during surgery (pathology) and the [stage of the cancer](#).

If you had [breast conserving surgery](#), your surgeon will tell you whether the [margins](#) around the cancer are clear. If the margins are not clear, you may need another operation. Sometimes cancer cells are found very close to the margin or in it. Although this can be upsetting news, it can usually be treated successfully with a second operation.

The surgeon and nurse will also talk to you about any further treatment you may need. This may be [radiotherapy](#), [chemotherapy](#), [targeted therapy](#) or [hormonal therapy](#). In some situations, your doctors may discuss having a [tumour-profiling test](#). This can help you decide about having chemotherapy.

About our information

References



Below is a sample of the sources used in our breast cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

ESMO. Early breast cancer clinical practice guidelines for diagnosis, treatment and follow-up. 2019, Vol 30, pp1192–1220. Available from: <https://www.esmo.org/guidelines/guidelines-by-topic/breast-cancer/early-breast-cancer> [accessed 2023].

National Institute for Health and Care Excellence (NICE). Early and locally advanced breast cancer: diagnosis and management. 2018. Updated 2023. Available from: <https://www.nice.org.uk/guidance/ng101> [accessed 2023].

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by [Dr Rebecca Roylance](#), Consultant Medical Oncologist and [Professor Mike Dixon](#), Professor of Surgery and Consultant Breast Surgeon.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information.](#)

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help